

Public Health Emergency Operations Center
"COUSP DRC"

MVE-17 Incident Management System

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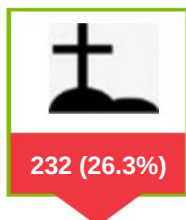
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°34/MVB_17/06/2026

Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (33)	• Ituri (21/36): Aru, Aungba, Bambu, Bunia, Damas, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia and Fataki. • North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. • South Kivu (1/34): Miti-Murhesa
Reporting date	June 17, 2026
Publication date	June 18, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



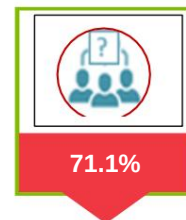
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 3 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and structures case management.
continuous harmonization and consolidations.

Suspicious deaths occurring in the community are subject to investigation.
further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

- **Twenty-one (21) new confirmed cases including 6 deaths**, were reported in the provinces of **Ituri (16 including 4 deaths) and North Kivu (5 including 2 deaths)** during the day of June 17, 2026. These confirmed cases are distributed from the health zones of **Bunia (4), Mongbwalu (4), Nizi (2) Lita (2), Komanda (1), Mangala (1), Nyankunde (1), Kilo (1)** in the province of Ituri as well as those of **Beni (2), Katwa (2) and Butembo (1)** in the province of North Kivu.
- **Eleven (11) patients** with Ebola virus disease have been declared cured after receiving negative control tests.
- **56 potential probable cases currently being validated** : Bunia (**18**), Rwampara (**15**), Nyankunde (6), Mongbwalu (6), Aru (3), Katwa (4), Lita (1), Mangala (1), Mambasa (**1**) and Miti-Murhesa (1). *A probable case is defined as a death occurring in a suspected case who did not undergo biological sampling (swab), but who has an epidemiological link to a confirmed case or a documented chain of transmission.*
- ***Data updated following the database cleaning and harmonization process DHIS2, including the consolidation of information from health zones, laboratories and case management structures. This process is ongoing.**

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

The Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The area

The Mongbwalu health center, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, the Ituri province had already been affected by the 10th Ebola virus (EV) epidemic, which was then raging in North Kivu. The current epidemic, caused by Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total population estimated at nearly 15 million inhabitants, with massive internal displacement and cross-border flows to neighboring countries.

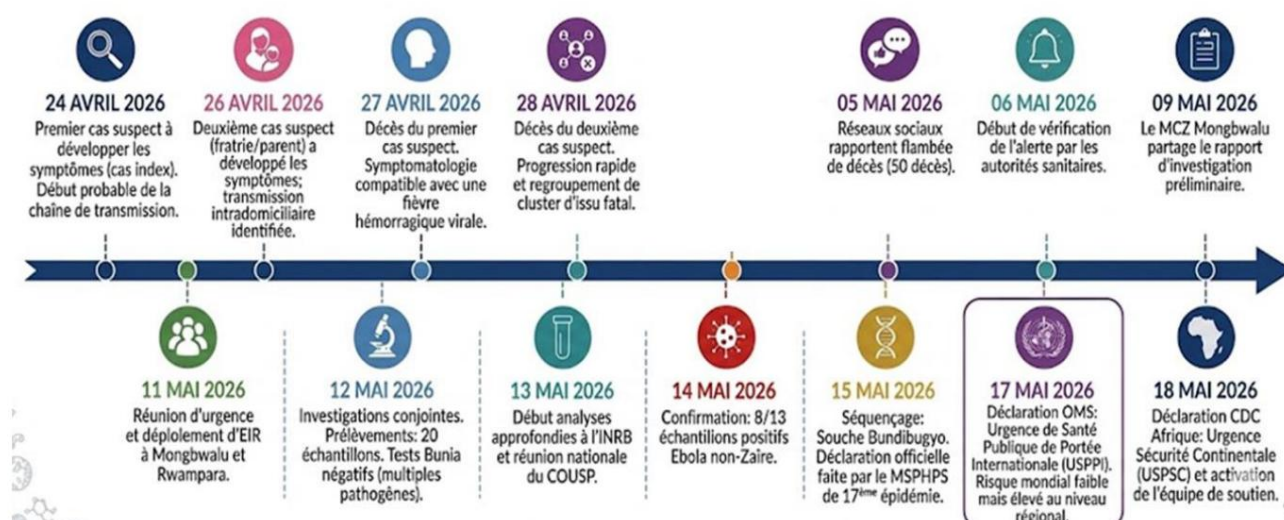


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 17, 2026

Table 1. Distribution of confirmed cases and deaths by affected province as of June 17, 2026.

Province	Confirmed cases	Deaths	Lethality	Affected health zones	New case confirmed (24h)
		(confirmed)			
Ituri	817	186	22.8%	21 out of 36 (58.3%)	16
North Kivu	76	45	59.2%	11 out of 34 (32.3%)	5
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	896	232	26.3%	33 out of 104 (31.7%)	21

For June 17, 2026, no new health zones were affected. The number of health zones

The proportion of affected provinces remains at 33/104 (section 3.1). Ituri province remains the epicenter, accounting for 91.2% of confirmed cases reported across the three affected provinces, two of which (Ituri and North Kivu) have remained active over the past 21 days. North Kivu province remains the most affected in terms of case fatality rate (59.2%), while Ituri province shows a low case fatality rate (22.8%) compared to North Kivu, despite the significant number of deaths recorded. Furthermore, South Kivu province has not reported any confirmed cases since May 26, 2026 (Table 1). There has been no recent transmission in South Kivu, but vigilance is still necessary.

3.2 Distribution by health zone (Cumulative as of 18/06/2026)

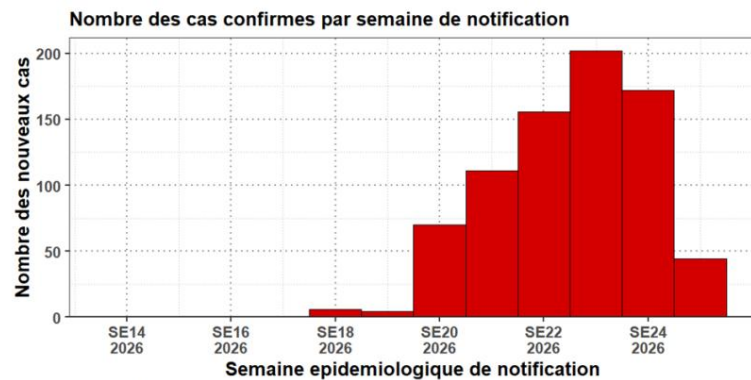
- **Ituri Province (817 cases):** Bunia (247), Rwampara (195), Mongbwalu (189), Nyankunde (68), Nizi (25), Lita (18), Komanda (10), Bambu (9), Kilo (7), Mangala (5), Tchomia (5), Damas (4), Aungba (4), Rimba (3), Aru (3), Mambasa (2), Logo (2), Drodro (1), Gety (1), Kambala (1), Nia-Nia (1) and Fataki (1).

Seventeen (17) additional cases are awaiting precise breakdown by health zone in the province.

- **North Kivu Province (76 cases):** Butembo (25), Katwa (24), Beni (15), Oicha (3), Kalunguta (2), Kyondo (2), Goma (1), Masereka (1), Vuhovi (1), Mabalako (1) and Musienene (1).

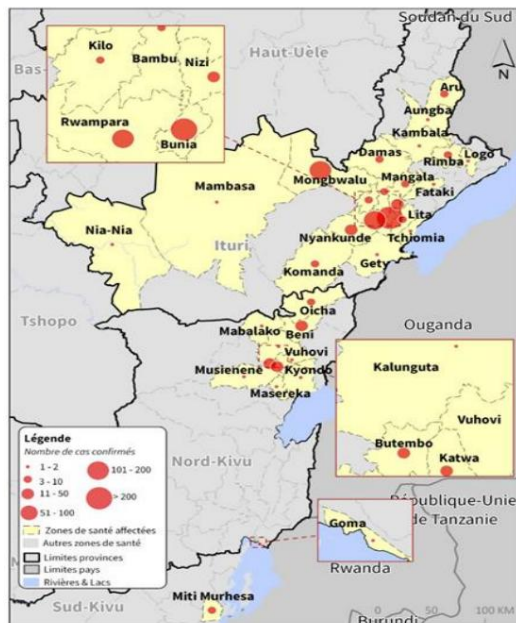
South Kivu Province (3 cases): Miti-Murhesa (3).

3.3 Temporal Analysis: An increasing number of confirmed cases is observed from one week to the next, reflecting continuous transmission of the disease within the community. Rapid geographical expansion is also observed. The epidemic remains possible if public health actions are not implemented quickly.



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses dans l'analyse : n cas confirmés = 765.

3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of June 17, 2026

Table 2: Confirmed cases and deaths by province and health zone (June 18, 2026)

Province / Health Zone	Cumulative confirmed cases (n)	Deaths among confirmed cases (n)	Case fatality rate (%)
ITURI	817	186	22.8%
Bunia	247	41	16.6%
Rwampara	195	33	16.9%
Mongbwalu	189	83	43.9%
Nyankunde	68	6	8.8%
Nizi	25	5	20.0%
Lita	18	4	22.2%
Komanda	10	3	30.0%
Bamboo	9	2	22.2%
Kilo	7	1	14.3%
Mangala	5		60.0%
Tchomia	5	30	0.0%
Damascus	4	0	0.0%
Aungba	4	1	25.0%
Rimba	3	0	0.0%
Aru	3	1	33.3%
Mambasa	2	1	50.0%
Logo	2	0	0.0%
Drodoro	1	1	100.0%
Gety		0	0.0%
Kambala	11	1	100.0%
Nia-Nia			100.0%
Fataki	11	10	0.0%
Other areas not yet identified	17	0	0.0%
NORTH KIVU	76	45	59.2%
Butembo	25	11	44.0%
Katwa	24	16	66.7%
Blessed	15	12	80.0%
Oicha	3	2	66.7%
Kalunguta	2	1	50.0%
Kyondo	2	1	50.0%
Goma	1	0	0.0%
Masereka	1	0	0.0%
Vuhovi			100.0%
Mabalako	11	10	0.0%
Musienene		1	100.0%
SOUTH KIVU	1	1	33.3%
Miti-Murhesa	33	1	33.3%
GRAND TOTAL	896	232	26.3%

As of June 18, 2026, the epidemiological situation of the 17th Ebola Virus Disease (EVD) epidemic in the Democratic Republic of Congo is characterized by a cumulative total of **896 confirmed cases** and **232 deaths**, resulting in an overall case fatality rate of **26.3%**.

Provincial dynamics and case concentration

The province of **Ituri** remains the absolute epicenter of the epidemic, accounting for **91.2%** of cases (n=817) and **80.2%** Deaths (n=186). The provincial case fatality rate is **22.8%**. The most affected health zones are Bunia (247 cases), Rwampara (195 cases), and Mongbwalu (189 cases). While Bunia and Rwampara have moderate case fatality rates (16.6% and 16.9%, respectively), the **Mongbwalu** health zone shows marked severity with a rate of **43.9%** (83 deaths out of 189 cases), suggesting persistent challenges in early management or access to care. Notably, 17 additional cases were reported in as-yet-unidentified health zones, without associated deaths, which could indicate a geographic expansion requiring further investigation.

Severity of the epidemic in North Kivu

North Kivu province presents the most concerning profile in terms of severity. With **76 confirmed cases** and **45 deaths**, the provincial case fatality rate reaches **59.2%**, more than double the national average. This high fatality rate is driven by the health zones of **Beni** (80.0%), **Katwa** (66.7%), and **Oicha** (66.7%). The Butembo area, although having a higher cumulative case count (n=25), has a slightly lower case fatality rate (44.0%). The presence of cases with a 100% case fatality rate in areas with low case numbers (Vuhovi, Musienene) is an indicator of the virus's danger in this region and the need for an immediate clinical response.

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri:

- Holding of the workshop to review the first actions of the response to Ebola virus disease.
- Holding of the coordination meeting on monitoring the activities carried out in the field by the partners technical and financial aspects across the pillars of the response.
- Held a strategic meeting with the new military governor of Ituri province.

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of June 17, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Alerts raised		266	755	7	1028
Alerts investigated		81	746	3	830
Investigation rate		30.5%	98.8%	42.9%	80.7%
Alerts confirmed	Alive	60	54	2	116
	Deceased	16	19	0	35
Total suspected cases for today		76	73	2	151

At the end of the day on June 17, 2026, **1,028 alerts** were reported in the three affected provinces, **830 (80.7%)** of these alerts were investigated, and **151 suspected cases (35 deaths)** were identified.

Table 4. Contact tracing of confirmed cases as of June 17, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri	4,659	3,297	70.8%
North Kivu	1,628	1,148	70.5%
South Kivu	80	80	100.0%
Total	6,367	4,525	71.1%

- Continued in-depth investigations and exhaustive listing and monitoring of contacts around confirmed cases in the ZS (narratives of confirmed cases, number of contacts listed and monitored per case).
- 319 new contacts were listed in 3 health zones in Ituri province. These contacts are distributed across the health zones of Bambu (168), Bunia (142) and Nizi (9).

Update on checkpoint and point of entry (PoC/PoE) activities

- PoC Mudzibala/Bunia: 1 alert (dead body) intercepted and notified. After document verification, the body was released for burial after a swab test was performed, and the result came back negative for Ebola Virus Disease (EVD); 1 live alert intercepted, notified, and validated in Mudzibala and referred to the Bunia General Referral Hospital (HGR).
- Bunia-Aru axis: 1 alert notified and validated at the PoC. Watsa road: 1 alert (dead body) intercepted and notified. After verification of documents, the body was released for the cemetery; 1 Alert notified and validated at the Ofoo PoC: 1 alert (lifeless body) intercepted, notified after verification of documents, the body was released for the cemetery with the swabs carried out being negative for Ebola Virus Disease (EVD).

Table 5. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC) in Ituri province, as of June 17, 2026

Indicators	Results
Proportion of activated PoCs	78.1% (43/55)
Number of travelers who passed	62,104
through, % of travelers screened,	96.0%
% of travelers who washed their hands, % of	93.8%
travelers made aware of the risks	91.0%
Number of alerts notified	4
Percentage of alerts investigated or bodies swabbed within 24 hours	100.0%
Number of validated alerts	4
% of results available in 24 hours	75.0%
Number of alerts transferred to the CT/CTE or for EDS	4
Number of contacts intercepted today	0

4.3 Laboratory

- **Ituri:** 126 samples collected and analyzed (positivity 12.7%; n=16).
- **North Kivu :** 70 new samples collected and analyzed (positivity 7.1%; n=5).
- **South Kivu :** 2 new samples collected and analyzed, all came back negative.

4.4 Infection Prevention and Control (IPC)

Ituri: Briefing of 34 service providers was organised in the Mambasa and Nyankunde health zones on standard precautions (correct hand washing, wearing and removal of masks, bio-cleaning); provision of ambulances in the general referral hospitals of the health zones (Komanda and Bunia); provision of Wash kits in the Mongbwalu and Bunia health zones.

- **North Kivu:** Assessment of health personnel exposed to a confirmed case of Ebola at Kitatumba General Referral Hospital: 3 providers were deemed high risk and placed in isolation; awareness-raising and briefing of 50 health providers on the main IPC themes in the context of the Ebola epidemic in the Health Zones of Beni, Butembo and Katwa; continuation of the training of 40 providers of the MATANDA Ebola Treatment Center under construction by the IMC; technical support and supervision of 32 community health workers in the health zones of Beni, Butembo, Katwa, Goma and Karisimbi; decontamination carried out in health facilities and households around confirmed cases of Ebola in Beni, Butembo and Katwa; completion of scorecard assessments in 7 ESS, including 3 traditional-modern structures in Katwa (Centre Tradi Yahwe RAPHA 15%, Source de Vie 5%, Dispensaire Mavondo 16%), 4 ESS in Beni (CH St Charles 48.8%, CS Butanuka 44%, CS Kasabinyale 51.1% and CS Ruwenzori 60.4%), representing an average of 43.2%; Closing of the briefing of health providers from MONUSCO clinics in Goma, Bunia and Kinshasa (65 participants) with the support of WHO.

4.5 Holistic care

- **Eleven (11) patients** with Ebola virus disease have been declared cured after obtaining negative control tests.
- Continued nutritional monitoring of patients in all CTEs.

Table 6. Patient movement within healthcare facilities (CTE/CT/CI) as of June 17, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	310	46	ND	356
Total admissions (24h)	61	6	ND	67
Exits (24h)				
Deceased (Suspects/Confessions)	7	0	ND	7
No case	14	4	ND	18
Healed	11	0	ND	11
Escapees (Suspects/Confessions)	3	0	ND	3
Transferred to the HGR	1	0	ND	1
Total outflows	36	4	ND	40
Patients in isolation (End of Day),	335	48	ND	383
including confirmed cases	143	18	ND	161
(NC+AC) and suspected cases	192	30	ND	222
Overall occupancy rate	99.1%	67.6%	N / A	93.6%

ND: Not available NA: Not applicable

Mental health activities and psychosocial support

Table 7. Key indicators of mental health and psychosocial support activities as of June 17, 2026

KEY INDICATORS	ITURI	SOUTH KIVU	NORTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	10 (55.5%)	4 (50%)	0
Confirmed cases currently being monitored that have received interventions SMSPS	41 (32.8%)	11 (37.9%)	1 (100%)
New suspected cases that benefited from SMSPS interventions	17 (27.8%)	4 (44.4%)	5 (100%)
Suspected cases under follow-up that have received interventions SMSPS	51 (26.5%)	15 (23.4%)	5 (100%)
Laboratory results have been announced.	22	4	0
Including positive ones	7	8	0
Number of children separated in daycare who benefited from interventions SMSPS	3	4	0
Number of children separated in the community who benefited from SMSPS interventions	0	9	1
Number of accompanying persons at the CTE who benefited from interventions SMSPS	40	32	28
Number of PPLs who benefited from SMSPS interventions	3	12	0
Number of household and community members who benefited from SMSPS interventions	44	12	0
Number of EDS supported	4	0	0

4.6 Continuity of care

- Reflection on the reorganization of the CPN in the context of the MVE.
- Recovery plan for social and solidarity economy enterprises already assessed.

4.7 Risk communication and community engagement

North Kivu: 25,160 people (15,113 women and 10,047 men) were sensitized in 12,922 households by 1,631 RECOs out of the 2,960 planned; 12,124 people were sensitized in Karisimbi, 7,654 in Goma and 5,382 in Nyiragongo, but 3 health areas in Karisimbi and 2 in Nyiragongo remain partially covered, which motivated the launch of mobile caravans with loudspeakers planned from June 19, 2026; 394 people were reached in school, university and religious settings, and 82 community actors (31 men and 51 women) were trained in Karisimbi on rumor management and the use of the SMS channel for alerts; 45 peer supporters were briefed through collaboration between the Provincial Health Division, WHO and MONUSCO.

Ituri: Community dialogue on mass awareness-raising of 2,275 affected individuals, including 901 men and 1,374 women, on compliance with Ebola prevention measures, care, community alerts, and EDS; home visits to 3,047 households by 394 Recos with messages on Ebola prevention measures and reporting of alerts (deaths and survivors) affecting 10,610 people (5,966 men and 4,644 women) in the health zones of Rwampara, Aru, Bunia, and Nyankunde; 303 Recos conducted educational talks in 41 households, reaching 3,073 people (1,342 men and 1,731 women); briefing of 100 Recos in the Kilo health zone on community-based surveillance; training on Dignified and Safe Burial (DSB) carried out in the health zones of Bambu and Nizi; dissemination of spots and messages in 18 different media on preventive measures for Ebola Virus Disease (EVD); hosting of programs on 4 media outlets on preventive measures for EVD.

Support for other pillars : Surveillance : listing of 6 contacts around a suspected case (Mongbwalu Health Zone, Shari Health Area); support for the investigation in Shari Health Area, Ngana, Tchanga, Hoho (Rwampara Health Zone). **IPC :** Preparation of a family for enrollment in the DHS (Rwampara Health Zone, Shari Health Area. Result: DHS completed) and decontamination of 2 households.

4.8. Logistics

- Receiving and delivering a refrigerator, some equipment and blood bags to the Provincial Centre of blood transfusion.
- Delivery of PCI inputs to the health zones of Nyankunde, Rwampara, Mongbwalu and Nia-nia.
- Delivery of dignity kits to the Rwampara Health Zone.

4.9. Security

- The security situation remains calm but unpredictable in Bunia and surrounding areas.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

Ituri : continued briefings on PSEA and good conduct in the community.

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, and strengthen team security.
2 Insufficient capacity in CTE and saturation in Ituri (96.4%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTE, deploy additional temporary structures
3 Poor performance in contact tracing (71.6% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4. Insufficient surveillance (33% of alerts investigated in Ituri)	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5 confirmed cases not localized or unclassified (~350 cases)	Major gaps in the reconstruction of transmission chains	Conducting a data reconciliation Remove any duplicates, and reclassify or specify the health zone and status (deceased, recovered, active, etc.). Verification active in the field
6 Insufficient supply of MEG and medical supplies	Weakening of overall care	Advocacy and urgent supply of essential medicines
7 Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (75 cases) HCW, lethality 22.6%	Urgent supply, strengthening of IPC training
8 Stock of samples not yet analyzed (backlog) at Laboratory in North Kivu and late diagnosis	Delayed confirmation, high lethality (58.9%)	Deploy laboratory catch-up plan, strengthen diagnostic capabilities

9	Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10	Insufficient and weak coordination use of COUSP frames	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, operationalize the COUSP
11.	Insufficient financial resources (gap of 200 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12	Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13.	Low increase in alerts (particularly in South Kivu)	Delay in case detection	Strengthening community surveillance and alert systems
14.	Continuity of Essential Services (CEHS) Compromised	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
15.	High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

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